



FOR THE ATTENTION OF

Dr. Chito Nwana

Arabella Women's Health

Asokoro, Abuja

Phase One: Revised Engagement Proposal

Phased Scope · Hybrid Commercial Model · Execution-First

Prepared by Consult For Africa · Healthcare Transformation and Management



00 - WRITTEN AROUND YOUR RESPONSE

We Read Your Response. This Proposal Is the Answer.

Your response confirmed three things that matter: you see the commercial opportunity, you agree that execution is the gap, and you believe the outcomes justify the investment. What you asked for is a structure that matches your cash position, a clear answer on the HMO transition, and a commercial model where both sides carry the risk of performance together. This revised proposal is built around those three asks specifically.

What has changed from the original proposal: The scope is now phased - Phase 1a focuses on foundation and execution. Phase 1b activates the revenue channels once the operational base is stable. The commercial model is hybrid: a reduced base retainer covering the embedded team, plus a performance fee tied to measurable revenue growth tracked through HospitlOS. PR and marketing activation is funded as a percentage of revenue growth. HERcast is named as a specific channel within that budget. Kola Momodu is named as C4A's Project Owner on-site from day one.

01 - THE BUSINESS CASE

What This Programme Generates

A conservative model built on Arabella's specific procedure mix. These figures do not assume a rebrand surge. They assume a well-executed commercial programme running consistently from Month 3 onward.

Revenue Driver	What Drives It	Conservative	Base Case	Optimistic
Premium Maternity Packages	5 to 8 full-package bookings/month at NGN 1.5M to 3.5M. One premium delivery every 4 days is achievable for a well-marketed specialist centre in Asokoro.	NGN 8M	NGN 16M	NGN 28M
MIS Gynaecology Surgery	7 to 15 MIS procedures/month. Volumes grow from Month 3 as the GP and physio referral network becomes active.	NGN 4M	NGN 7M	NGN 14M
Fertility & IVF / ART	3 to 6 IVF/IUI cycles/month. Fertility demand in the FCT is severely underserved. Once Arabella has a clinical reputation, the referral pipeline is self-sustaining.	NGN 3M	NGN 10M	NGN 20M
Tabitha Reactivation	8 to 15% conversion of 3,000+ database over a 3-month sprint. Fastest and cheapest revenue in Phase One.	NGN 2M	NGN 4M	NGN 8M
Corporate & HMO Referrals	Embassy belt, multinationals, oil and gas FCT. Referrals compound from Month 2 as HMO panels are confirmed and corporate retainer agreements are signed.	NGN 1.5M	NGN 3.5M	NGN 8M
Wellness, Medspa & Nutrition	Weight management, IV therapy, aesthetics, Cookedindoors nutrition. Low acquisition cost; high repeat visit rate from existing patients.	NGN 1M	NGN 2.5M	NGN 6M



Pharmacy Revenue (VMI uplift)	Medbury VMI normalises stock, aligns formulary to clinical mix. Pharmacy from cost centre to margin contributor from Month 2.	NGN 0.5M	NGN 1.2M	NGN 2.5M
TOTAL MONTHLY REVENUE UPLIFT		NGN 20M	NGN 44.2M	NGN 86.5M
MONTHLY C4A BASE RETAINER (Month 4+)		NGN 2.2M	NGN 2.2M	NGN 2.2M
PERFORMANCE FEE (% of growth above baseline)		variable	variable	variable
NET MONTHLY RETURN (after base retainer)		NGN 17.8M	NGN 42M	NGN 84.3M

Note on performance fee: The performance fee is calculated on monthly revenue above the diagnostic baseline confirmed in Week 1. Both sides agree the baseline before any invoice is raised. The net return figures above show the position after the base retainer only.

02 - THE HMO TRANSITION STRATEGY

Arabella Is a New Entity. Here Is Exactly What C4A Does.

Arabella Women's Health is a new CAC-registered legal entity with its own RC number, entirely separate from Tabitha Medical Centre. This is confirmed. C4A does not apply for HMO panels from scratch. The approach is a structured successor provider introduction: clinical continuity documentation, a formal introduction letter from Dr. Chito as the lead clinician, and direct engagement with HMO medical directors through C4A's existing relationships at Hygeia, AXA Mansard, Avon HMO, Reliance, and others. Where tariff portability is possible, C4A negotiates for it. Where it is not, C4A uses the introduction as a negotiation moment rather than a cold application.

Arabella Women's Health is a new CAC-registered entity. C4A's HMO approach is a structured successor provider introduction, not a fresh application from scratch. Clinical continuity documentation links Arabella to the Tabitha patient base. C4A's direct relationships with HMO medical directors at Hygeia, AXA Mansard, Avon HMO, and Reliance are used to negotiate tariff portability. The transition moment is also a tariff improvement moment. In most cases Arabella will end this process with better tariffs than Tabitha held. We do not just protect what you have. We improve it.

What we need from you before we begin: The Arabella CAC certificate of incorporation and RC number. A list of current Tabitha HMO panels with tariff rates where available. Dr. Chito's signed introduction letter (C4A prepares the draft). C4A handles everything from there.

03 - THE DELIVERY TEAM

Specific People. Specific Roles. Embedded From Day One.

One of the consistent themes in your response was execution. Not strategy documents, not frameworks - execution. The team structure below is built around that. Every named person has a specific accountability. There is no ambiguity about who does what.

**Kola Momodu****C4A Embedded****Credentials & Track Record**

- ✓ 8 years 10 months at Cedarcrest Hospitals, Abuja - Software Administrator and Senior IT Support
- ✓ Managed HMS, PACs server, Odoo, Active Directory across multiple branches - 99.8% uptime
- ✓ Developed custom software to close workflow gaps and improve efficiency by 25%
- ✓ ISO/IEC 27001:2022 Lead Auditor certified (April 2026) - patient data governance
- ✓ Scrum Fundamentals certified - structured delivery methodology
- ✓ Power BI - data visualisation and management reporting
- ✓ Outstanding Staff Award 2018, Cedarcrest Long Service Award 2023

C4A Project Owner (On-Site) - Technology, Operations Analytics & Execution

Kola is physically present inside Arabella Women's Health as C4A's Project Owner on-site. 5 days/week. His primary accountability is execution: making sure what is supposed to happen actually happens, and proving it with data. He deploys and manages HospitlOS, owns the weekly data brief every Monday, conducts the IT and EMR audit in Week 1, and maintains the operational data infrastructure that the performance fee is calculated against. Independent contractor - WHT 5% deducted at source and remitted to FIRS by C4A.

C4A Seconded Clinical Operations Lead**C4A Secondment****Credentials & Track Record**

- ✓ Senior clinical professional - postgraduate clinical qualification
- ✓ Track record in private sector hospital quality and governance
- ✓ Accreditation pathway experience (SafeCare, COHSASA, or JCI)
- ✓ Specific candidate confirmed and briefed before engagement start

Head of Quality & Clinical Operations - 3 days/week (Months 3 to 6), 4 days from Month 7

A senior clinical professional from within C4A's own network, embedded in Arabella on a fixed-term secondment as Head of Quality and Clinical Operations. Owns clinical governance, patient safety systems, accreditation readiness, and the clinical quality framework across every service line. Reports directly to you as part of the Arabella leadership team. On-site schedule agreed at kickoff.

Dr. Debo Odulana - Account Principal**C4A Leadership****Credentials & Track Record**

- ✓ MBBS, University of Sharjah · MSc International Health Management, Imperial College London
- ✓ Former CEO, Cedarcrest Hospitals Abuja
- ✓ Former Chief Innovation & Strategy Officer, Evercare Hospital Lekki
- ✓ Founder, Doctoor Health (acquired by Evercare)
- ✓ Active HMO relationships: Hygeia, AXA Mansard, Avon, Reliance, and others

C4A Founder & CEO - 1 day/week Phase 1a, 2 days/week Phase 1b

Overall strategic accountability for the Arabella engagement. Relationship principal, HMO network lead, and the connection to C4A's broader platform: HospitlOS, Maarova, Medbury, and Cookedindoors. Monthly strategic review with you directly. HMO introductions and BD relationship management at senior level led by Debo personally.

Note on Tolu: Your Chief of Staff continues in her existing role. Kola's remit is the C4A project workstream - technology, data, and execution accountability. He works alongside Tolu, not in place of her.

04 - PHASE 1A: FOUNDATION FIRST (MONTHS 1 TO 2)**Fix the Engine Before Pressing the Accelerator.**

Your response was clear: the gap has not been strategy. It has been execution. Running marketing spend into an operation without task accountability, pharmacy discipline, and financial visibility generates noise, not



revenue. Phase 1a is built around this. It addresses the operational foundation first, before any marketing is activated. Your cash commitment in this phase is at its lowest.

Work Item	Timing	Cost	What Happens
Operations & Commercial Diagnostic	Week 1 to 2	Complimentary	Kola and C4A spend the first two weeks inside Arabella: current revenue by payer, billing process and leakage points, pharmacy stock and formulary gaps, IT infrastructure, staff task compliance, and Tabitha patient database quality. Written report delivered by end of Week 2. Establishes the performance baseline for the performance fee. No invoice.
HospitlOS Deployment	Week 1 to 2	Free - 6 months	Kola configures and deploys HospitlOS in Week 1. All patient-facing staff trained by end of Week 2. Task management, shift handovers, billing triggers, pharmacy restock alerts, and the management dashboard go live before any marketing spend is activated.
IT & EMR Audit	Week 1 to 2	Complimentary	Comprehensive review of existing IT infrastructure: EMR, ERP, accounting, POS, network, website, patient portal, and data security posture. ISO 27001 lens applied. Written report with prioritised implementation roadmap within 14 days.
Pharmacy VMI - Medbury Onboarding	Week 2 to 4	Included	1 day/week. Medbury VMI agreement established. Formulary audit completed against Arabella's clinical mix: obstetrics, gynaecology, fertility, wellness. Restock alerts integrated into HospitlOS.
HMO Transition Management	Week 1 to 6	Included	C4A prepares and submits the successor provider introduction package to all existing Tabitha HMO panels. Tariff portability negotiations initiated. Status of each panel tracked in HospitlOS and reported to Dr. Chito weekly.
Tabitha Database Segmentation	Week 2 to 4	Included	3,000+ registered patients segmented by condition type, procedure history, recency, and spend level. Prioritised outreach list prepared for the Month 3 reactivation sprint. The list does not move until the operational foundation is in place to receive patients properly.
C4A Clinical Secondment Embed	Week 1 onwards	Included	3 days/week. Clinical governance framework scoped. Patient safety protocols reviewed. CX training programme begins for patient-facing staff by Week 3.

Phase 1a principle: The diagnostic is free. HospitlOS is free for six months. The base retainer in Months 1 and 2 is at its lowest. The operational gains in this phase generate returns that fund Phase 1b.

05 - PHASE 1B: REVENUE ACTIVATION (MONTH 3 ONWARDS)

The Marketing Engine. Now With Something to Run Into.

By Month 3, the operational foundation is in place. HospitlOS is live. The pharmacy is managed. The HMO panels are confirmed or in process. The Tabitha database is segmented. The clinical quality framework is operating. Now the revenue activation begins.

Fixed Items (Enabling Functions)

**01 C4A Strategy & Account Management (full)**

2 days/week - monthly business review, weekly brief, strategy adjustment, performance fee reporting, dedicated account lead.

02 C4A Clinical Secondment - Head of Quality

3 days/week (Months 3 to 6), increasing to 4 days from Month 7 - clinical governance, patient safety, CX training, quality improvement, accreditation readiness.

03 Digital Marketing & Patient Funnel

Google Search and Meta campaigns targeting premium FCT women's health terms, WhatsApp booking funnel with automated follow-up, geofencing across 13 named Abuja target zones. All channels tracked to HospitlOS.

04 GP & Physiotherapy Referral Network

Formal referral relationships with FCT GPs, physiotherapy practices, and diagnostic centres. Referral portal tracks source, volume, and conversion.

05 Pharmacy & HR Management (expanded)

1 to 2 days/week - Medbury VMI monitoring and performance reporting, outsourced HR management via Maarova (contracts, appraisals, policy).

Performance-Linked (Paid on Results Only)

No conversion or signing, no charge on these items.

06 Tabitha Reactivation - Commission Based (Month 3 to 5)

8% of first procedure or package revenue from each reactivated Tabitha patient converted during the sprint window. Tracked by patient source tag in HospitlOS. No conversion, no charge.

07 Corporate & HMO BD - Success Fee

NGN 200,000/month base (Months 3 to 6) covers active engagement effort. NGN 150,000 success fee per signed corporate retainer. 5% of first quarter HMO panel revenue per new panel onboarded. BD base drops after Month 6.

08 PR, Media & HERcast

Monthly media pitching, press releases, broadcast placements for you. HERcast is a named channel within the marketing activation budget - content calendar integrated, PR amplified across all channels. Marketing activation funded as 5% of monthly revenue growth above baseline.

06 - HERCAST**An Asset Most Clinics Would Pay to Build.**

HERcast is Dr. Chito's podcast. It is not a substitute for the PR and marketing programme. It is an accelerant. A podcast with an established audience of women already interested in women's health and quality of life is exactly the clinical authority channel that most practices spend months and significant budget trying to create from nothing. Arabella already has it.

■ Existing Audience

HERcast has a built and engaged listener base. These are women who have chosen to seek out this content. Arabella does not need to pay to reach them.



■ Dr. Chito as Clinical Voice

Episode content positions Dr. Chito as the specialist authority on women's health in Abuja: fertility, MIS gynaecology, maternal health, wellness. Clinical insight that builds genuine trust. Conversion from listener to patient is higher than any paid channel.

■ Content Calendar Integration

HERcast episodes planned alongside the C4A content calendar. An episode on fertility runs when the fertility pipeline workstream is active. Content and commercial activation are synchronised.

■ PR and Media Amplification

Each HERcast episode becomes a PR asset: a press release hook, a LinkedIn post, a WhatsApp broadcast. Content created once and distributed across every channel.

■ Budget Allocation

HERcast is allocated a named portion of the marketing activation budget (5% of monthly revenue growth above baseline). It scales with the business - not capped at a fixed monthly line.

07 - THE HYBRID COMMERCIAL MODEL

You Heard This in Conversation. Here It Is Formally.

A pure revenue-share is difficult to structure cleanly in a clinical setting. A pure retainer puts all the risk on Arabella before results are visible. The hybrid model solves both: a reduced base retainer covers the embedded team and core execution, a performance fee captures C4A's share of growth, and the marketing budget scales automatically with revenue. Both sides carry the risk and share the upside.

Component	What It Covers	Structure	When Paid
Base Retainer	Kola Momodu on-site, C4A clinical secondment, HospitlOS, account management, strategy oversight, pharmacy VMI management	Phase 1a (Months 1 to 2): NGN 1,775,000/month Phase 1b fixed (Month 3+): NGN 1,350,000/month Ongoing (Month 6+): NGN 2,925,000/month	Monthly in advance
Performance Fee	C4A's share of monthly revenue growth above the diagnostic baseline established in Week 1	Tiered: 8% on growth NGN 5M to 15M above baseline 10% on growth NGN 15M to 30M above baseline 12% on growth above NGN 30M above baseline	Monthly in arrears (month following achievement)
Marketing Activation Budget	Digital campaigns, geofencing, PR, social media, HERcast. Contracted directly by Arabella. C4A manages strategy and execution.	5% of monthly revenue growth above baseline (minimum NGN 400,000/month in Months 1 to 3) Scales automatically as revenue grows	Monthly - contracted directly by Arabella (outside C4A scope)

Worked example: Diagnostic baseline = NGN 8M/month. Month 4 total revenue = NGN 30M.

Growth: NGN 22M above baseline.

Performance fee: NGN 800,000 (8% of first NGN 10M) + NGN 700,000 (10% of next NGN 7M) = NGN 1,500,000.

C4A total Month 4: NGN 1,350,000 base + NGN 1,500,000 perf + NGN 200,000 BD base = NGN 3,050,000.

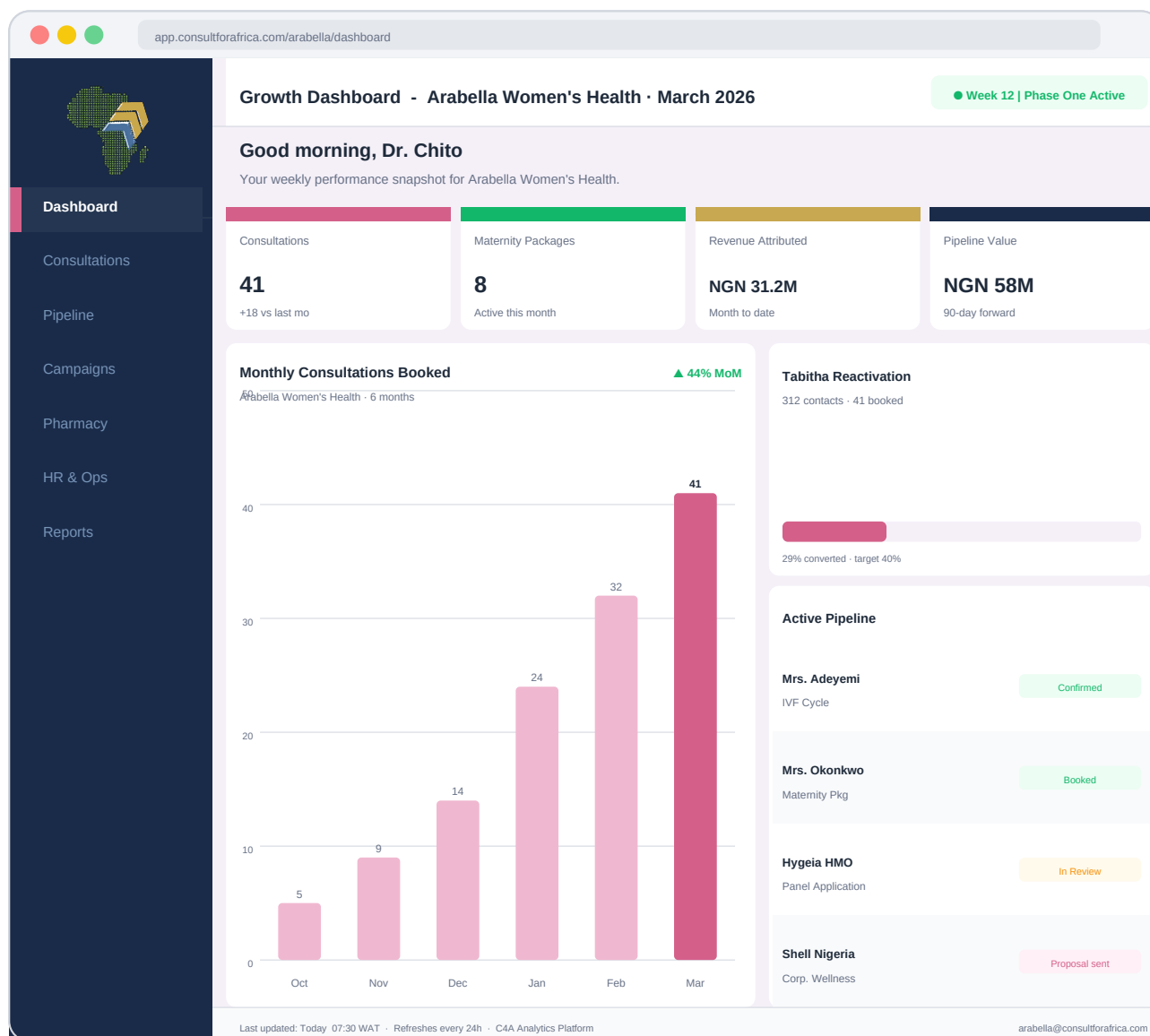
Arabella nets: NGN 26.95M. Marketing activation (5% of NGN 22M growth) = NGN 1,100,000 - contracted directly by Arabella.



08 - WHAT DR. CHITO SEES EVERY WEEK

A Real-Time Business Dashboard. Not a PDF Report.

C4A provides live access to Arabella's commercial and operational performance through the HospitlOS analytics dashboard. Kola maintains the data integrity. The performance fee is calculated from this dashboard.



Dr. Chito receives a weekly WhatsApp brief (5-minute read) every Monday and a monthly business review call with Debo covering full KPI analysis, budget vs. actual, channel performance, and strategic recommendations for the month ahead.

09 - COMMERCIAL & MARKETING STRATEGY

Strategy Before Spend.

Before a single ad is placed, C4A conducts a structured commercial and marketing strategy review. The rebrand from Tabitha to Arabella is a repositioning of a clinical business into a premium market segment it has never formally occupied. That requires a clear thesis, rigorous testing, and the discipline to adjust when the data tells you to.



01

Market & Demand Analysis

C4A maps the FCT women's health market: competitor facilities, pricing, service mix. Unmet demand assessed by procedure type. The market intelligence layer all decisions sit on.

02

Segmentation, Targeting & Positioning

Arabella serves specific segments with specific willingness to pay. C4A identifies and sizes the priority segments, builds a targeting priority map, and defines the positioning precisely. This is the hypothesis tested and refined throughout Phase One.

03

Pricing Strategy & Package Architecture

Procedure-level pricing benchmarked against the Abuja premium market. Package architecture designed to maximise lifetime patient value. Self-pay and HMO tiering built separately.

04

Geofencing, PR & Offline Presence

Precision digital geofencing across 20+ named Abuja locations - competitor hospitals, premium gyms, diplomatic zones, corporate campuses, Asokoro residential belt, and airports. Complemented by targeted PR placements in BusinessDay, Nairametrics, and Pulse Nigeria.

05

Patient Journey & Process Re-engineering

The full Arabella patient journey mapped from first search to post-procedure follow-up. Every friction point identified. Booking funnel, consultation conversion, billing interaction, and post-care follow-up redesigned before the marketing engine drives volume.

06

Hypothesis Testing & Market Validation

Every element is a testable hypothesis. C4A sets measurable targets, tracks weekly on the dashboard, and adjusts. Phase 1b is a structured market validation exercise, not a fixed campaign plan.

10 - INVESTMENT SUMMARY**What Arabella Pays. When. For What.****A. One-Time Setup**

Item	NGN
Patient Data Analysis & Segmentation (Tabitha database, 3,000+ records)	NGN 250,000
CRM & Outreach Tracking Setup (WhatsApp CRM, booking funnel, conversion tracking)	NGN 200,000
Corporate BD Target Account List & Materials (embassies, multinationals, oil & gas)	NGN 200,000
Digital Marketing Strategy & Account Setup (Google, Meta, geofencing architecture)	NGN 250,000
SEO Audit & On-Page Optimisation (keyword targeting, Google Business Profile)	NGN 250,000
PR Strategy, Media List & Press Materials (press kit, bio, two press releases)	NGN 200,000
Billboard Creative Design (2 sites - Asokoro + Maitama arterial corridor)	NGN 350,000
Pharmacy VMI Audit & Medbury Onboarding (formulary review, SOP development)	NGN 350,000



C4A Secondment Onboarding (induction, governance framework scoping, Week 1 embed)	NGN 300,000
Operations & Commercial Diagnostic - structured 2-week assessment, written report	Complimentary (market value NGN 600,000)
IT & EMR Audit - full written strategic report with prioritised roadmap	Complimentary (market value NGN 400,000)
HospitlOS Configuration & Full Staff Onboarding (Kola-led deployment)	Complimentary (market value NGN 500,000)
Arabella Website Redesign - new patient-facing site with booking integration	Complimentary (market value NGN 1,200,000)
GP & Physiotherapy Referral Portal - design, build and deployment	Complimentary (market value NGN 500,000)
Cookedindoors Integration	Complimentary (market value NGN 150,000)
TOTAL SETUP (excl. complimentary)	NGN 2,350,000
TOTAL VALUE INCLUDING COMPLIMENTARY	NGN 5,700,000

B. Monthly Management Fees (Hybrid Retainer)

Work Stream	What's Included	Monthly NGN
PHASE 1A - Foundation (Months 1 to 2)	Minimum viable. Fix the engine first.	NGN 1,775,000 / mo
C4A Strategy & Project Oversight	1 day/week - monthly business review, weekly check-in, HMO transition management, account oversight. Increases to 2 days/week from Phase 1b.	NGN 500,000
Kola Momodu - C4A Project Owner (On-Site)	5 days/week on-site - HospitlOS deployment and management, operational data integrity, weekly brief, IT infrastructure. Independent contractor - WHT 5% deducted at source and remitted to FIRS by C4A.	NGN 1,075,000
HospitlOS Monthly Subscription (FREE - Months 1 to 6)	Platform, hosting, updates, support, compliance reporting	FREE - Months 1 to 6
Pharmacy - Medbury VMI Coordination	1 day/week - formulary audit, VMI agreement setup, restock triggers in HospitlOS. Expands in Phase 1b.	NGN 200,000
	Fixed items: clinical secondment, account management, pharmacy.	NGN 1,350,000 fixed + perf fees
C4A Strategy & Account Management (full)	2 days/week - monthly business review, weekly brief, strategy adjustment, performance fee reporting, dedicated account lead.	NGN 400,000
C4A Clinical Secondment - Head of Quality	3 days/week (Months 3 to 6), increasing to 4 days from Month 7 - clinical governance, patient safety, CX training, quality improvement, accreditation readiness.	NGN 600,000



Pharmacy & HR Management (expended)	1 to 2 days/week - Medbury VMI monitoring and performance reporting, outsourced HR management via Maarova (contracts, appraisals, policy).	NGN 150,000
Performance-Linked (paid on results)	No conversion or signing, no charge.	Variable
Tabitha Reactivation - Commission Based	8% of first procedure or package revenue from each reactivated Tabitha patient converted during the sprint window (Months 3 to 5). Tracked by patient source tag in HospitlOS. No conversion, no charge.	8% of reactivation revenue
Corporate & HMO BD - Success Fee	NGN 200,000/month base (Months 3 to 6). NGN 150,000 success fee per signed corporate retainer. 5% of first quarter HMO panel revenue per new panel onboarded. BD base drops after Month 6.	NGN 200K base + success fees
PHASE 1A TOTAL (Months 1 to 2)	Kola on-site. HospitlOS live. Pharmacy managed.	NGN 1,775,000 / mo
PHASE 1B FIXED (Month 3+)	Clinical, management, pharmacy. Performance fees additional.	NGN 1,350,000 / mo + perf
ONGOING (Month 6+)	BD base drops. HospitlOS still free.	NGN 2,925,000 / mo + perf fees

Finance Team as a Service - Available on Request

A full embedded CFO is not the right fit at this stage. C4A makes its finance team available as a service - activated when Arabella needs it, not running as a fixed monthly cost.

Option	What's Included	Cost
Monthly Finance Package	Monthly management accounts, P&L; by service line, cash position report, HMO receivables tracking, and a 30-minute review call with Dr. Chito. Core package - activated when Dr. Chito is ready.	NGN 250,000 per month
Quarterly Board Pack (add-on)	Full quarterly board-ready financial pack: management accounts with commentary, budget vs. actuals analysis, service line performance, cash flow forecast, and a structured narrative for board or investor review.	NGN 150,000 per quarter (add-on to monthly)

C. Total Investment Summary

Component	Amount (NGN)
One-time setup (excl. complimentary)	NGN 2,350,000
Phase 1a retainer - Months 1 to 2 (Kola on-site, foundation only)	NGN 1,775,000 / month
Phase 1b fixed retainer - Month 3+ (clinical secondment, management, pharmacy, BD base)	NGN 1,350,000 / month
Phase 1b performance-linked (reactivation commission + BD success fees)	Variable - paid on results only
Ongoing retainer - Month 6+ (BD base expires)	NGN 2,925,000 / month + perf fees
Performance fee (% of monthly revenue growth - paid in arrears)	Variable - see Section 07



Marketing activation (5% of revenue growth - contracted by Arabella)	Variable - outside C4A scope
TOTAL MONTH ONE - Option C (setup deposit NGN 750K + Phase 1a retainer)	NGN 2,525,000
TOTAL MONTH ONE - Option A (full setup + Phase 1a retainer)	NGN 4,125,000
vs. original proposal Month One	Original NGN 9,100,000 - saving NGN 4,975,000

Payback context: The base retainer in Month One is recovered by a single premium maternity package or two MIS procedures. The performance fee is paid from revenue that does not exist today. There is no scenario in which Arabella pays C4A more than it earns from the programme.



11 - GETTING STARTED ON YOUR TERMS

The First Step Costs Nothing.

C4A begins with a complimentary operations and commercial diagnostic - two weeks inside the practice, no invoice, no obligation. The diagnostic establishes the performance baseline for the performance fee and delivers a written report that Arabella keeps regardless of whether the engagement proceeds.

Complimentary: Operations & Commercial Diagnostic

Kola and the C4A team spend two weeks inside Arabella. Revenue by payer, billing process, pharmacy performance, patient database quality, IT infrastructure, and HMO panel status all assessed. Written report delivered within 14 days. The baseline revenue figure confirmed for performance fee purposes. No invoice. No commitment. The report is yours to keep.

How to Start

Amount	What It Covers
NGN 750,000	Deposit at signing. Covers diagnostic launch and all setup preparation.
NGN 1,775,000 / month	Phase 1a retainer - Months 1 and 2. Kola on-site, HospitlOS live, pharmacy managed. Full setup balance spread across Months 2 and 3.
NGN 1,350,000 / month + perf	Phase 1b from Month 3. Clinical secondment embedded. Revenue activation begins. Performance fee on growth above baseline.
NGN 2,925,000 / month + perf	Ongoing from Month 6. Sprints complete. Programme runs on performance.

The diagnostic is complimentary. The deposit is NGN 750,000. Month One total cash out is NGN 2,525,000 - covered by a single premium maternity package. From Month 3, performance fees are paid only from revenue growth that did not exist before the programme started.

12 - DELIVERY TIMELINE

What Happens When

Phase	Timing	Milestone	What the Arabella Team Will See
Phase 1a	Wk 1 to 2	Foundation	Diagnostic begins. Kola on-site day one. HospitlOS deployed. IT audit commenced. Tabitha database segmented. HMO notification package prepared. Clinical secondment embedded.
Phase 1a	Wk 3 to 4	Operational stability	HospitlOS live across all departments. Medbury VMI agreement signed. HMO introduction letters submitted. CX training begins. First weekly data brief issued.
Phase 1a	Month 2	Revenue preparation	HMO transition status confirmed per panel. Tabitha outreach list finalised. Corporate BD target list approved. Digital strategy and account setup complete. IT audit report delivered.



Phase 1b	Month 3	Revenue activation	Tabitha reactivation sprint launches. Google and Meta campaigns go live. Corporate BD meetings begin. First converted Tabitha reactivation patient booked. Performance fee period begins.
Phase 1b	Month 4 to 6	Pipeline building	First HMO panel confirmed. First corporate retainer signed. GP referral network active. HERcast episode published. Fertility pipeline activated.
Phase 1b	Month 6+	Scale and optimise	Campaigns optimised on 90-day data. BD base drops. Ongoing retainer. Phase 2 scope - medspa, telemedicine, clinical governance accreditation - scoped and presented.

13 - BEYOND PHASE ONE

What Arabella Can Become

Medspa - Commercial Strategy & Launch

Strategy, Growth & Commercial

Weight management clinic, IV infusion therapy, aesthetics. High-margin brand extension.

Telemedicine - Arabella Anywhere

Strategy, Growth & Commercial

Diaspora patients returning, FCT patients who cannot attend in person, international patients. Telemedicine extends Arabella's reach without expanding physical capacity.

Clinical Governance & Accreditation

Clinical Governance & Quality

SafeCare, COHSASA, or JCI readiness. Required for premium HMO panels and international health programmes.

Financial Reporting & Fractional Finance

Financial Recovery & Reporting

Once HospitlOS has 6 months of clean data, a full institutional-grade financial reporting suite: investor-ready pack, service line P&L, pricing strategy review.

Arabella Lagos - Phase 3 Expansion

Strategy, Growth & Commercial

Once Arabella Abuja is stable, profitable, and properly staffed - C4A maps the Lagos opportunity. Get Arabella Abuja right first.

Transaction Advisory - Capital or Exit

Transaction Advisory

A clinic with formal HMO relationships, structured corporate retainers, and a proven premium brand is building institutional value. C4A advises on valuation and deal structuring.

14 - NEXT STEPS



14

The Next Step Costs Nothing.

The diagnostic is complimentary. Kola arrives in Week One. The performance baseline is established before any fee is invoiced. We can be inside Arabella within a week of your call.

Step 1	One Conversation	30 minutes with Debo. Confirm the HMO entity question. Agree diagnostic start date.
Step 2	Complimentary Diagnostic	Two weeks. Kola on-site. Written report and performance baseline delivered. No invoice.
Step 3	Choose Your Option	Option A, B, or C. Confirm scope. C4A mobilises Phase 1a within 48 hours.

Ready to confirm the mandate?

partnerships@consultforafrica.com

consultforafrica.com

Consult For Africa - Healthcare Transformation and Management · Lagos, Nigeria · 135+ Years Combined Senior Leadership · 6+ Active Engagements Across Africa